DevsHub.cc

DevsHub Imaging

Product Overview & Feature Guide



From imaging files to connected patient care.

A responsive web platform for centers, doctors and patients

Problem and solution

The problem behind a result handoff

An examination can leave the center as imaging files, a report and separate patient details. Staff arrange a transfer, explain who the files belong to and help the doctor find a suitable reader. The patient may separately request something printable. A repeat visit, unreadable file or mistaken recipient adds another round of coordination.

This is a representative manual journey, rather than a claim that every center follows the same process. Its practical gaps are familiar: similar patient names, several people in one archive, scattered visit history and uncertainty about which doctor can see which result.

DevsHub connects these steps. The center reviews identity and complete source ownership, the doctor receives usable examination access with sending-center context, and the patient receives a simple printable document. Files no longer have to carry the whole workflow by themselves.

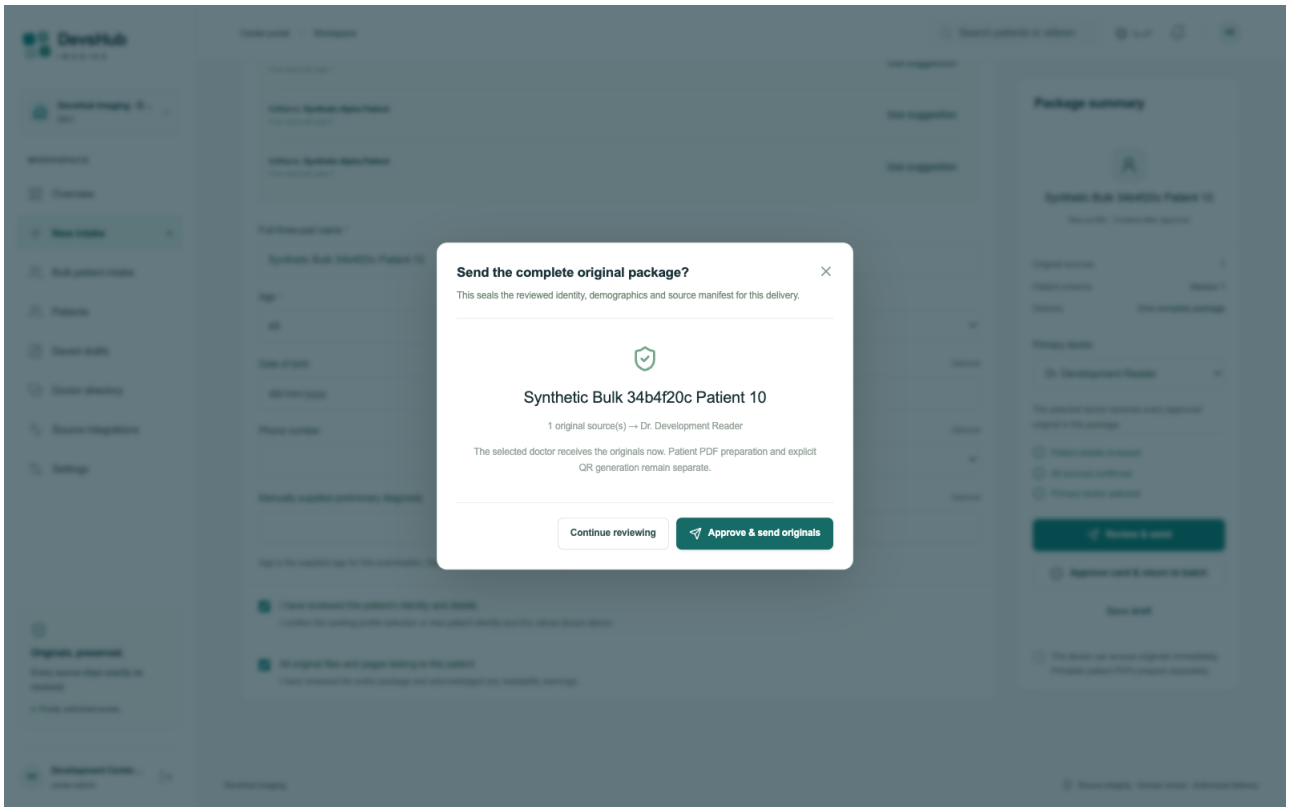
Problem and solution

One clear journey from upload to results

Start an intake or resume a saved draft. Upload the received files, review suggested demographics or enter missing information, and confirm an existing patient or a distinct new person. Check that every selected source belongs to that person, choose the permitted doctor and press Send.

The doctor receives the complete reviewed original package without waiting for PDF preparation or barcode generation. Readable imaging opens in the integrated reader; received PDFs open as documents. Unsupported originals remain visible with clear recovery guidance.

Printable output is prepared separately. When needed, the center deliberately generates the patient's persistent QR card, which opens one selected examination through a normal browser. The patient can preview, download and print PDF output. Center, doctor and administrator workspaces keep their responsibilities clear while sharing the same connected journey.



Final patient-package review, shown before sending demonstration records.

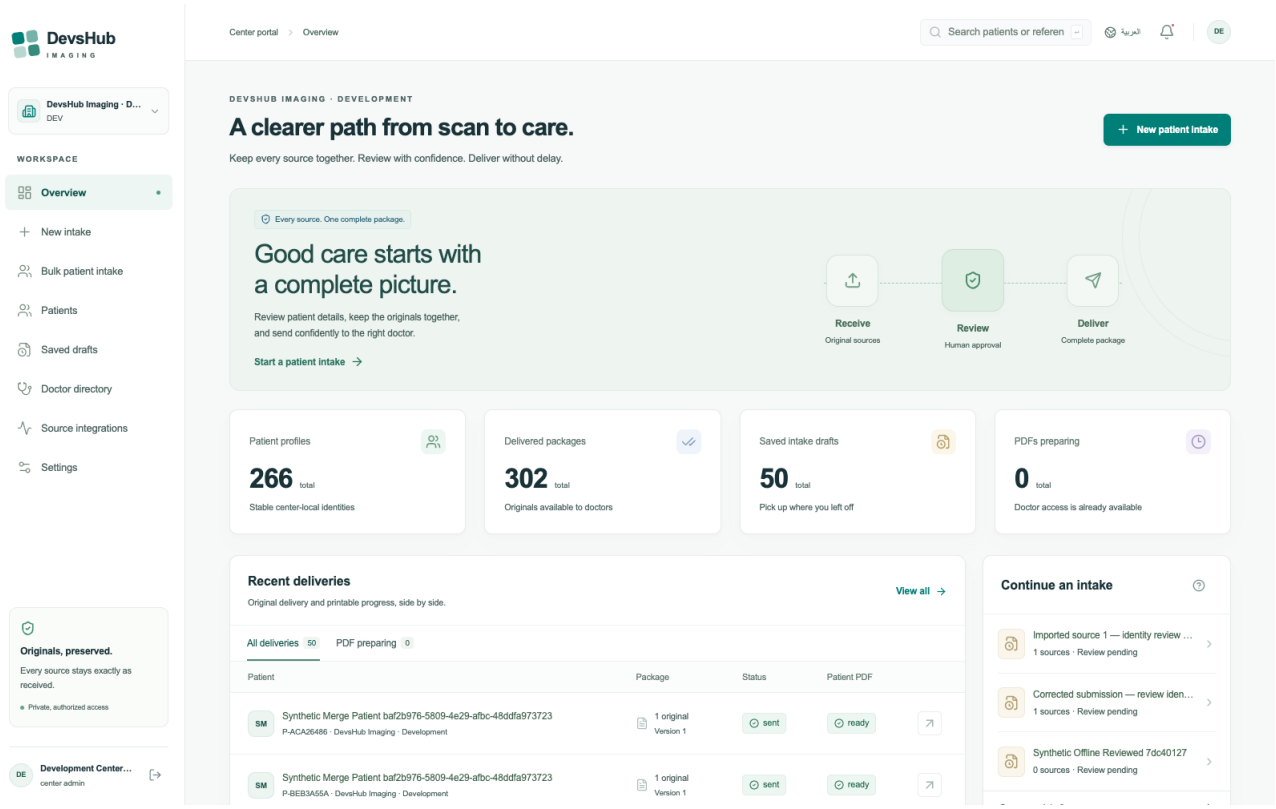
The center workflow

A center workspace that keeps work moving

The center dashboard brings together new intake, patient search, saved drafts and sent results. Staff can open earlier visits, identify unfinished work and follow source delivery separately from printable processing. Clear progress labels and short guidance explain the next action.

Several drafts can remain unfinished at once. Autosave retains staged fields, source references, patient and doctor choices and workflow progress. Authorized work can recover after refresh or interrupted connectivity, and large transfers can resume. If a file needs re-selection, the interface says so instead of claiming it was uploaded.

Autosave does not confirm a patient or send a result. Sending still requires current authorization and reviewed identity and sources. An unwanted draft can be explicitly deleted without deleting an already sent examination. Relevant in-app notifications remain accessible through the bell, with a brief banner linking to the result.



The center dashboard with demonstration records.

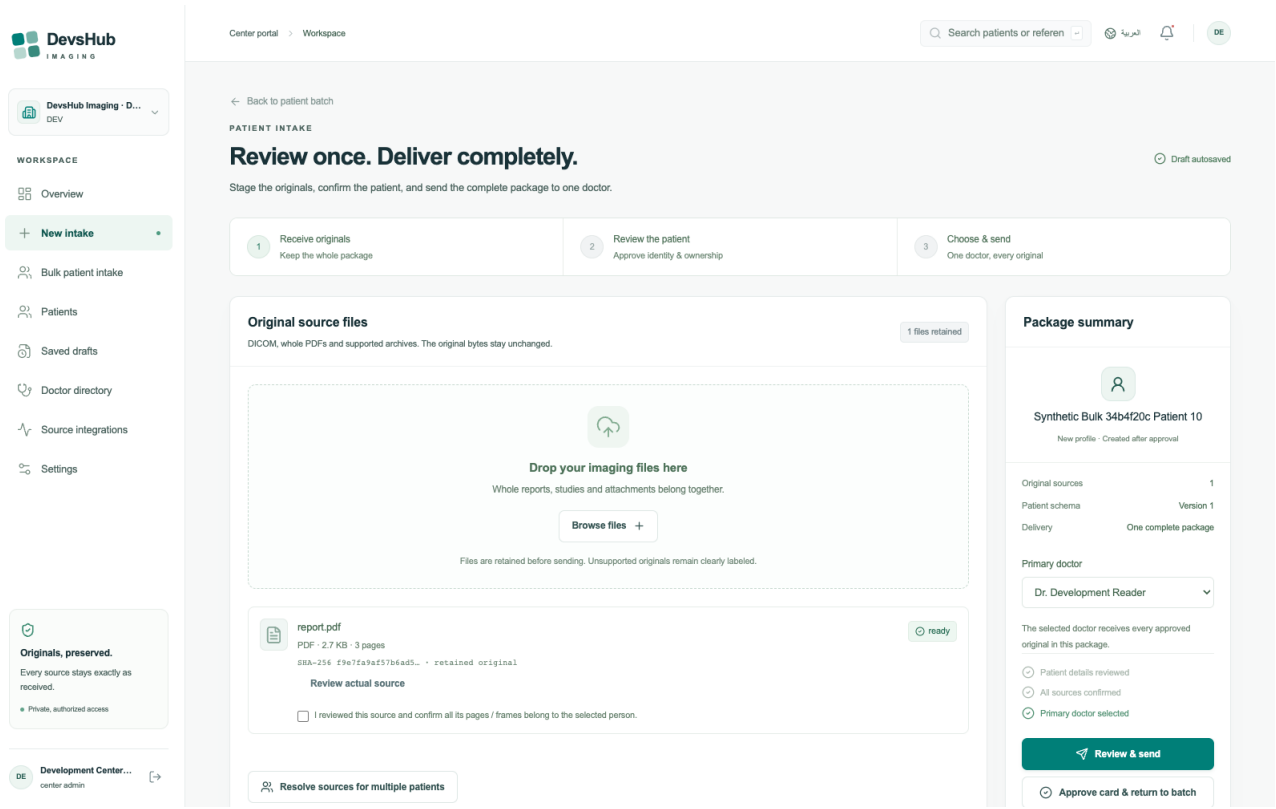
The center workflow

Imaging files and PDF reports, preserved

DICOM is the medical imaging format used by MRI and ultrasound equipment. Receive individual DICOM objects, files without a normal extension, ZIP archives and PDF reports. Intake checks actual content and keeps all selected current sources for one confirmed person together. A multi-person archive needs separate reviewed patient cards; a PDF containing different patients needs corrected patient-specific documents, not automatic ownership guesses.

Received reports retain their language, wording, pages and bytes. DICOM tags and pixels stay unchanged. Demographic extraction supplies reviewable suggestions; it never translates a clinical report, writes a diagnosis or decides patient identity. Approved profile values remain separate from the source.

Unreadable or unsupported files stay listed. With known ownership and explicit acknowledgment, they can still reach the doctor unchanged for a resend discussion. Unknown ownership remains pending. A rendering failure cannot silently remove a file or make incomplete printable output appear complete.



Source intake and review using demonstration records.

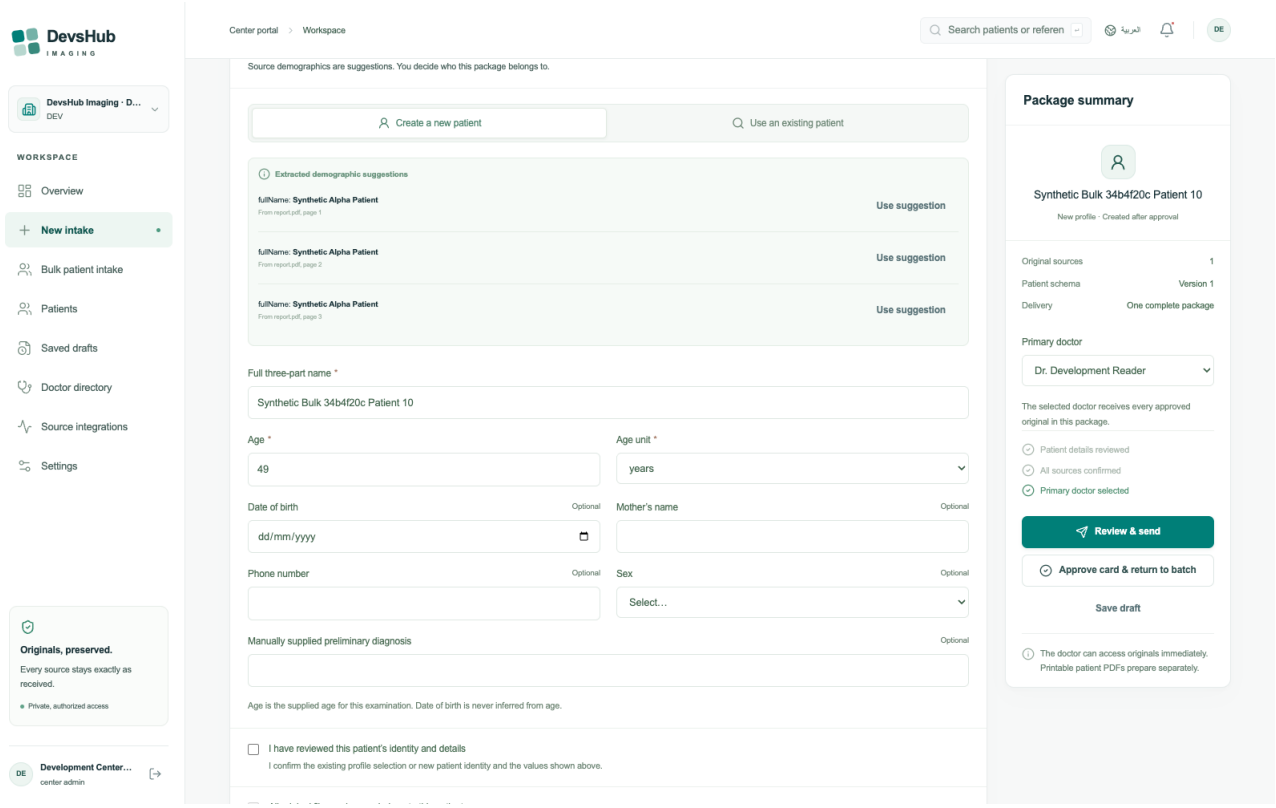
The center workflow

Confirm the person, preserve their history

Both imaging and PDF intake use a patient profile. Default required values are the full three-part name and supplied age. Exact birth date, two-part mother's name, phone, preliminary diagnosis and extra fields start as optional. Age keeps its recording context; the system never invents a birth date from it.

Search suggestions show distinct center-local profiles with stable patient codes and available comparison details. Equal names and ages never automatically select or merge people. Staff confirm the existing person or explicitly confirm a distinct new one; unresolved ownership stays pending. Extracted values remain suggestions until approved.

A returning person's new examination becomes a separate visit under the confirmed profile, preserving earlier snapshots. Staff may also explicitly save a reviewed profile before files arrive. Draft autosave creates no finalized patient or examination. Doctors see only visits they are currently authorized to open.



Explicit identity and whole-source review before sending.

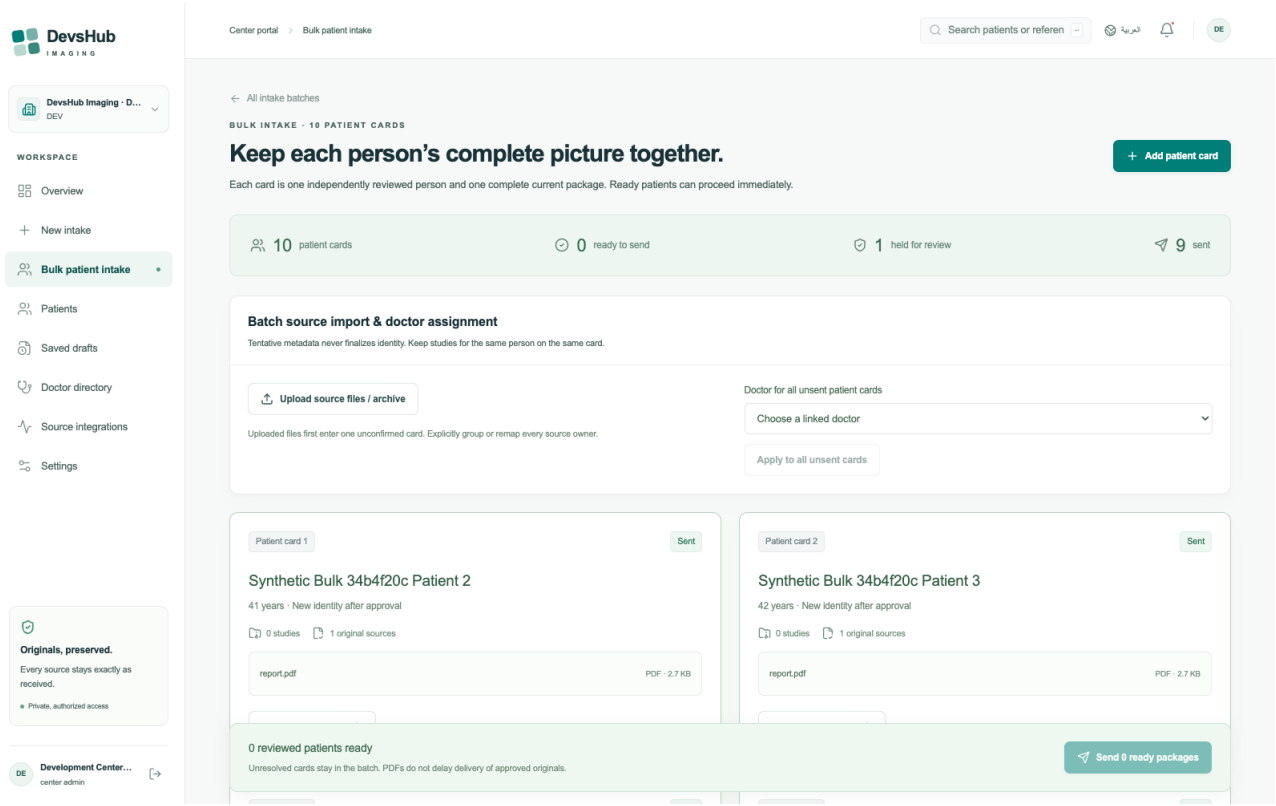
The center workflow

Bulk upload, individual patient control

Bulk review creates one card for each confirmed person, keeping that person's selected current studies and documents together. Staff can apply one doctor to the batch or override the doctor for each patient. Several studies belonging to one person are not treated as several patients.

If individual intake identifies multiple candidate source people, it can offer a switch to bulk review without re-uploading. Every card still needs reviewed demographics, explicit identity and complete source ownership. Ready cards can be sent while unresolved work remains held. Ten patients is an example, not a fixed limit.

Approval belongs to the reviewed content. Changing demographics, source membership or the effective field schema requires renewed review before Send. Lost-response retries recover the same accepted dispatch rather than duplicating it. A deliberately submitted new visit remains a distinct patient package.



Patient cards with shared and individual doctor choices.

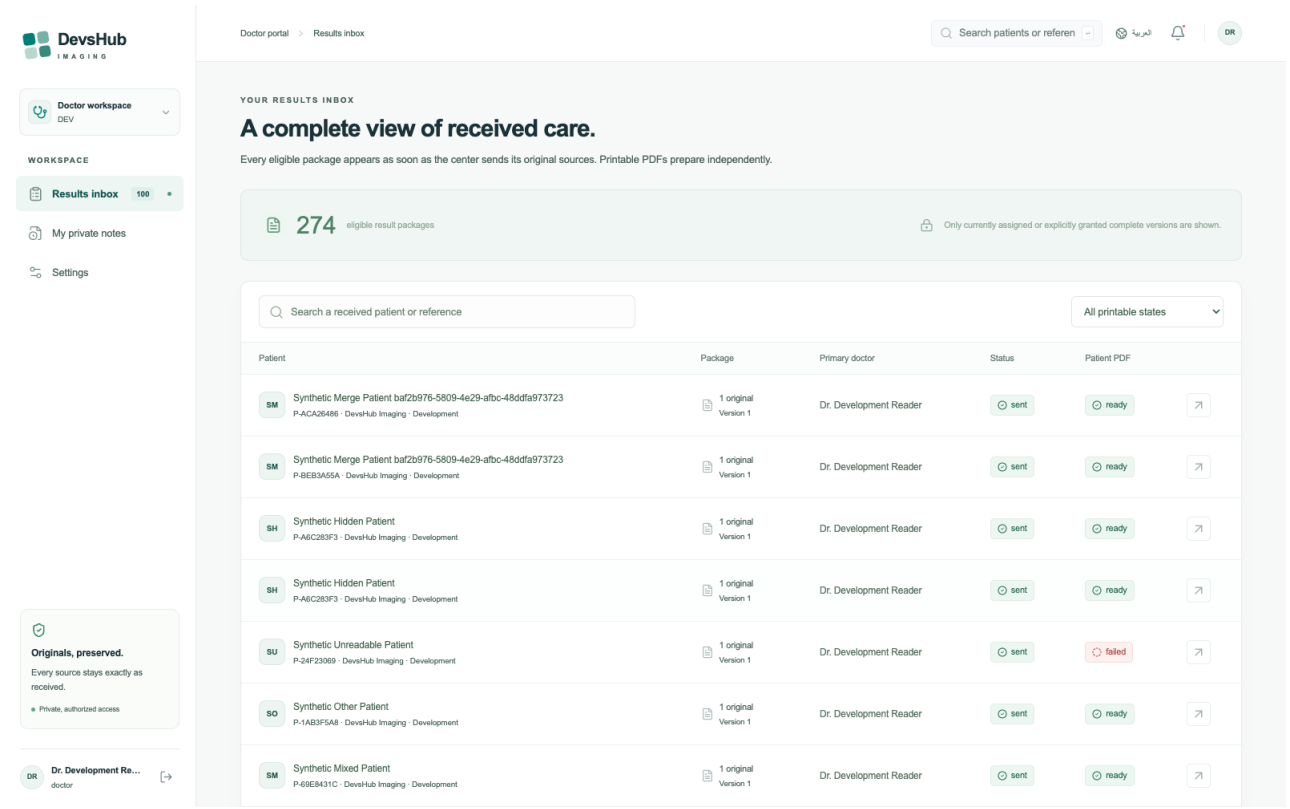
The doctor workspace

The doctor's inbox, with the right context

Doctors search, filter and browse currently authorized results with the sending center, patient context, source package and printable status visible. Originals are available after Send while PDF preparation continues independently. Relevant notifications lead directly to the examination without an external messaging requirement.

The doctor's patient workspace brings together eligible examinations and their own note history, keeping visit dates, sources and permissions distinct. Receiving a later visit does not automatically reveal earlier visits. Future or unassigned results stay outside access, including after a profile merge.

Permissions apply throughout the journey: inbox, search, profile headers, history, readers, notes and downloads. Hidden or revoked results do not remain available through an old file link. A doctor working with several centers retains clear sending-center context rather than gaining unrestricted access to those centers' patients.



The populated doctor inbox with eligible demonstration results.

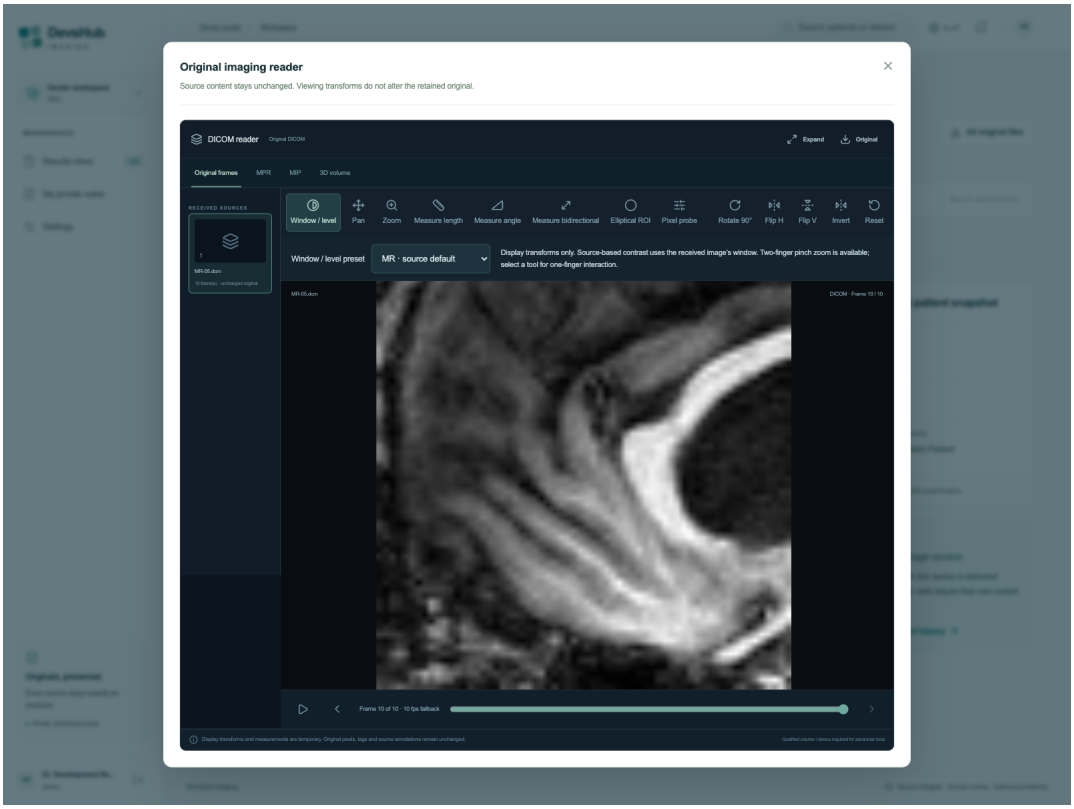
The doctor workspace

MRI and ultrasound in one integrated reader

Open source images, select series, navigate frames and use cine when the examination provides it. Display controls include window/level, pan, zoom, rotation, flipping, presets and reset. Temporary Length, Angle, Bidirectional, ROI and Probe tools support inspection without editing the original examination or publishing annotations.

Suitable spatial series can use MPR, linked crosshairs, MIP and 3D volume views. These modes depend on the received geometry, supported decoding and available browser resources. Original frames remain available when a volume view is unsuitable. A supported sample does not mean every scanner export has identical compatibility.

The same web reader adapts to phones, tablets and desktops with labeled touch tools and frame controls. Larger screens provide more room for detailed spatial work. There are no native mobile apps; users open the web application, with unsupported states shown clearly instead of hiding sources.



Original MRI viewing with a sample examination.

The doctor workspace

Ultrasound without changing the workflow

Ultrasound examinations follow the same reviewed journey as MRI: the center confirms the person and complete package, selects the permitted doctor and sends the originals. The doctor can open supported ultrasound images in the integrated reader, navigate the available frames and use cine when the source provides it.

The examination keeps its sending-center context and patient association. Display and temporary measurement tools help the doctor inspect the received images, while original files remain unchanged and available for download. A received PDF ultrasound report opens in the ordinary document reader and preserves its original language and pages.

Supported viewing depends on the actual export format. If a member cannot be displayed, the doctor still sees the retained file and its status, and can contact the center for a usable copy. Nothing disappears silently, and the patient continues to receive printable PDF output.



Ultrasound frame viewing with a sample examination.

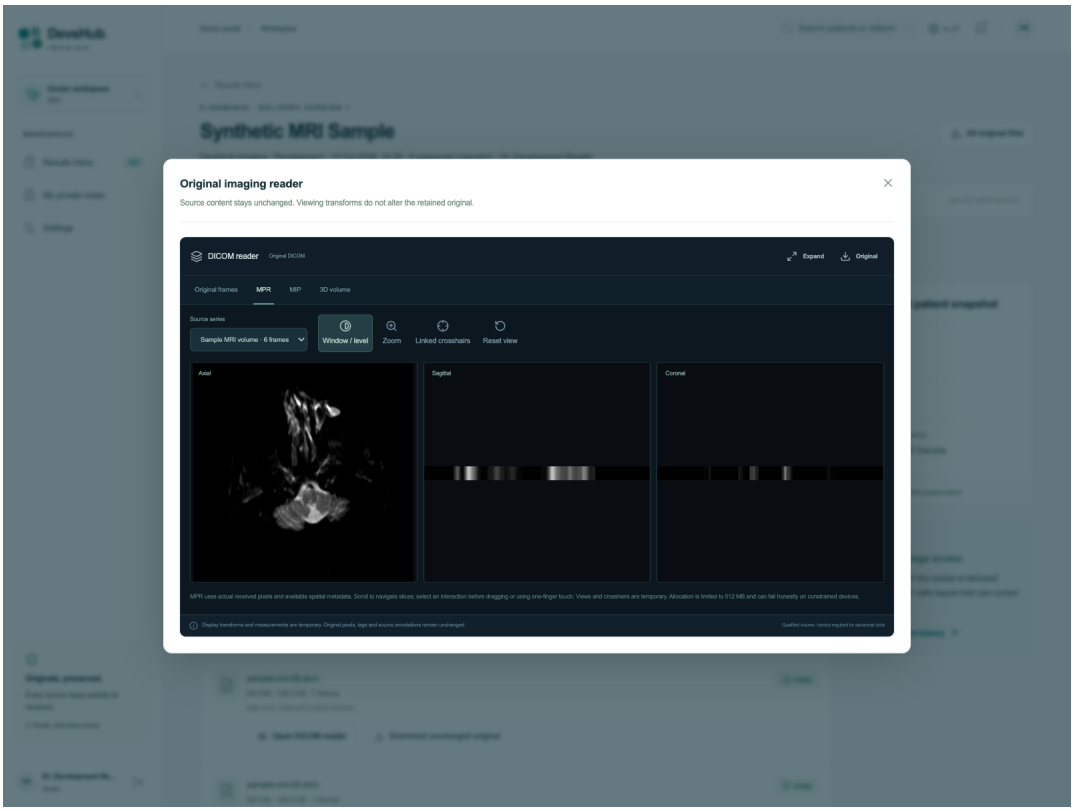
The doctor workspace

More ways to explore suitable series

For compatible spatial series, multiplanar reconstruction (MPR) presents views through the same image volume in different planes. Linked crosshairs keep those views aligned around a selected position. Maximum intensity projection (MIP) offers a projection view, while 3D volume viewing provides another way to explore the same received examination.

These modes are display tools. They do not rewrite source pixels, publish shared annotations or replace original frame access. Their availability depends on the received geometry, supported decoding and browser resources. If a volume view is unsuitable, the system explains the limitation and retains the ordinary source images.

The web interface also provides labeled touch controls on smaller screens. Phones are useful for access and basic inspection; a laptop or larger display gives more room for detailed spatial work. Patients receive the simpler printable PDF experience rather than this imaging workspace.



Multiplanar views and linked crosshairs using sample spatial images.

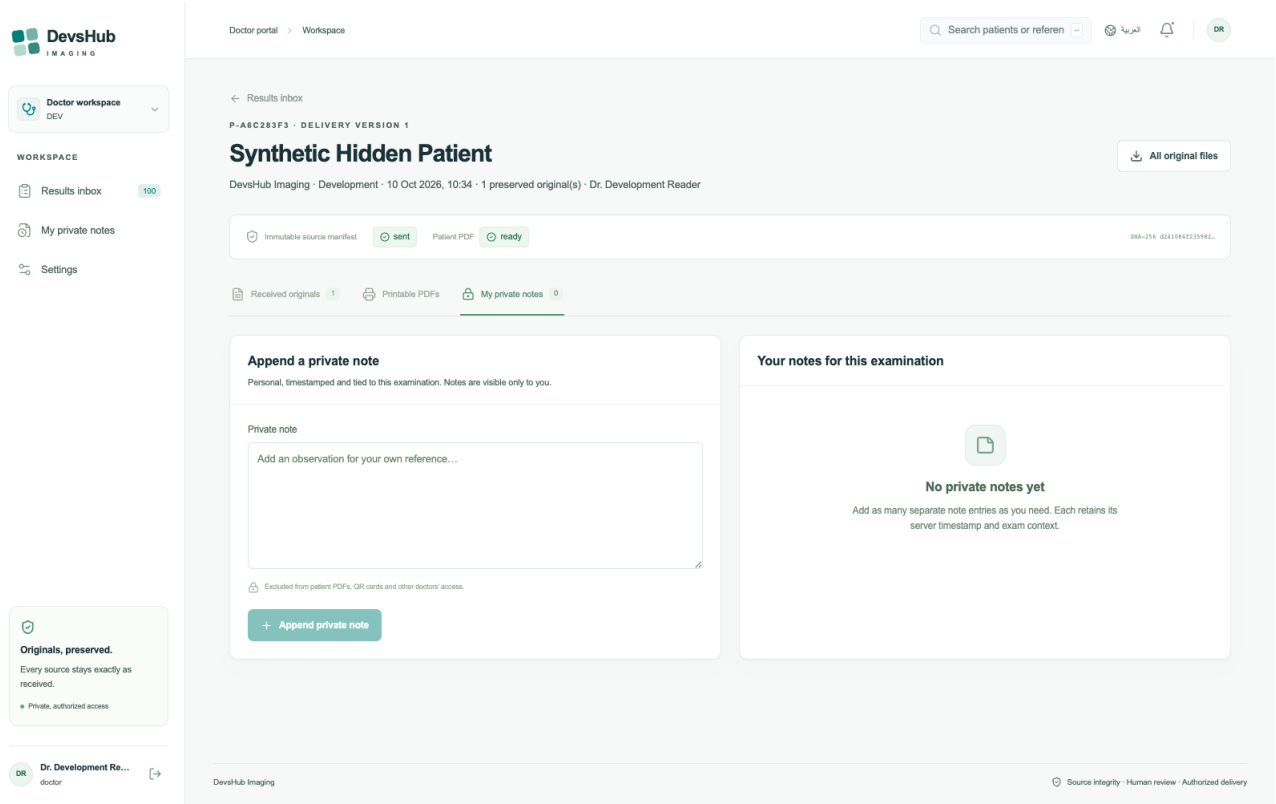
The doctor workspace

Private notes and trustworthy downloads

Doctors can append multiple personal notes to an eligible examination, each carrying the author and server date/time. Their own chronological notes remain separate from received reports, source files and patient PDFs. Other doctors and patients cannot read them, even after sharing or merging. The platform administrator retains audited global access; expressly permitted note administrators remain scoped.

Eligible doctors can download original members, a patient-specific ZIP with an integrity manifest and complete printable output. Received PDFs use ordinary viewing, printing and download controls. Current permission is checked again when serving a file.

A multi-person parent ZIP is never sent wholesale to one ordinary doctor. Only that confirmed patient's authorized members are bundled, preserving their bytes and excluding unrelated names, files and indexes. A safe single-person archive may remain unchanged. Patient links offer PDF only, never raw DICOM, source archives or private notes.



The private note-entry form with an empty own-note state.

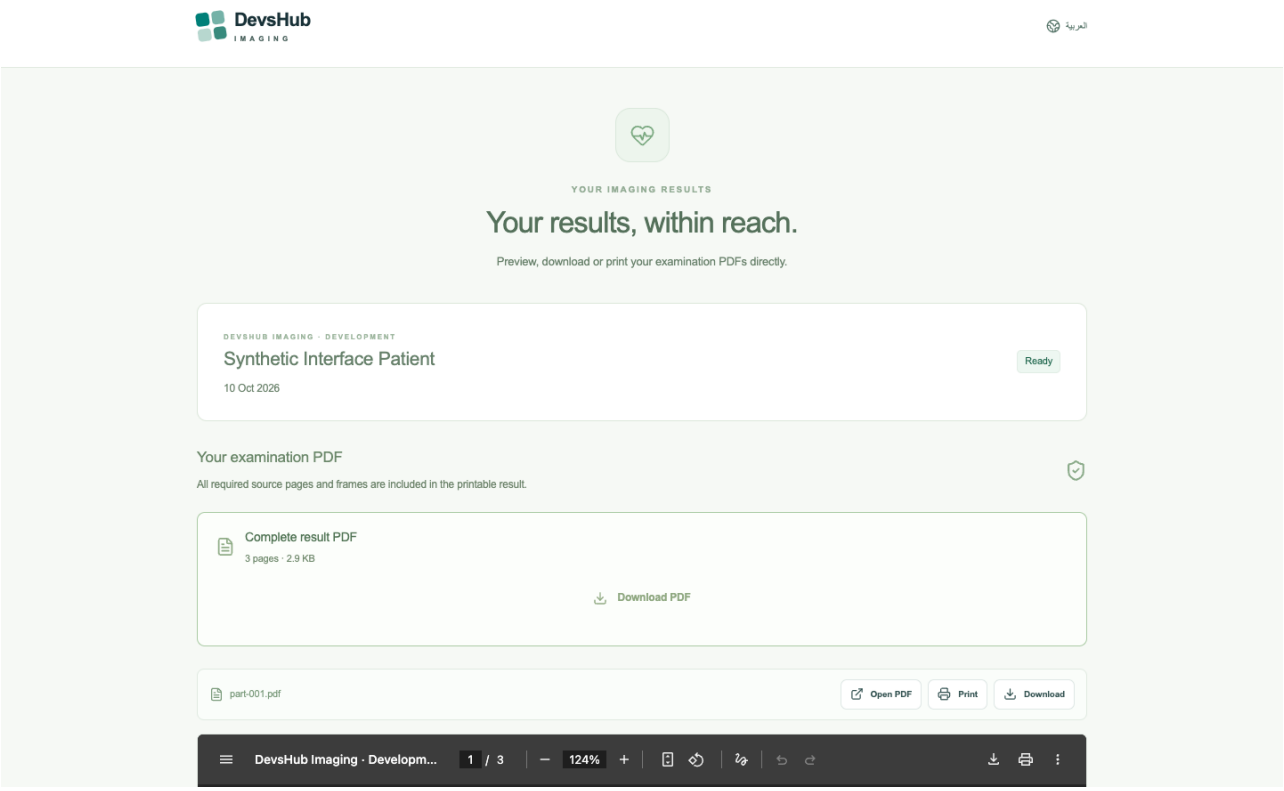
The patient experience

Patient results: simple PDF and persistent QR

After Send, staff deliberately generate a QR card for one selected sent examination. Its unpredictable link is not a patient ID, phone or sequential number, and it never opens the patient's whole history. While active, it works across devices without patient login, PIN, OTP or expiry. A lost card can be downloaded or printed again; withdrawal, explicit revocation or deletion can deny access.

The patient page shows examination details and ready PDF preview before download. Patients print or download a complete PDF or all ordered, numbered parts. No imaging archive, raw-file route or doctor note is offered.

Printable output preserves supported report pages, attachments and required frames. Missing or unreadable content stays explicitly incomplete. Branding never overlays received report pages. A static PDF does not reproduce every cine or interactive 3D function, so doctors retain original-reader access separately.



The patient result overview with demonstration records.

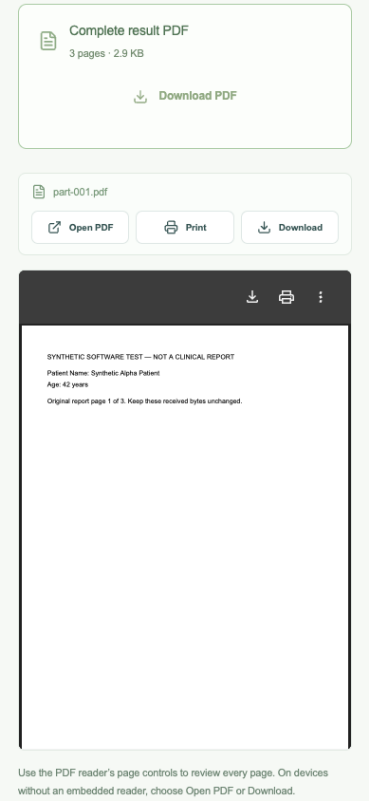
The patient experience

Phone access and a simple WhatsApp handoff

Patients open the same result page through a normal phone browser, preview ready PDF output and download or print the complete document or numbered parts. There is no patient account setup or staff administration on this page. Preparing or unreadable-source states explain when printable output is not yet complete and when to contact the center.

For WhatsApp sharing, staff download the QR card as PDF or image and open the patient's chat when an optional usable number is available. On desktop, they use WhatsApp Web in the same browser, manually attach the card and press Send.

The product does not automatically attach files, send messages or claim WhatsApp delivery confirmation. No extension is required. If a popup is blocked or no number is available, staff can open the chat manually or print/download the card. Missing phone information never blocks the doctor's result or patient-card printing.



The patient PDF page at a phone viewport.

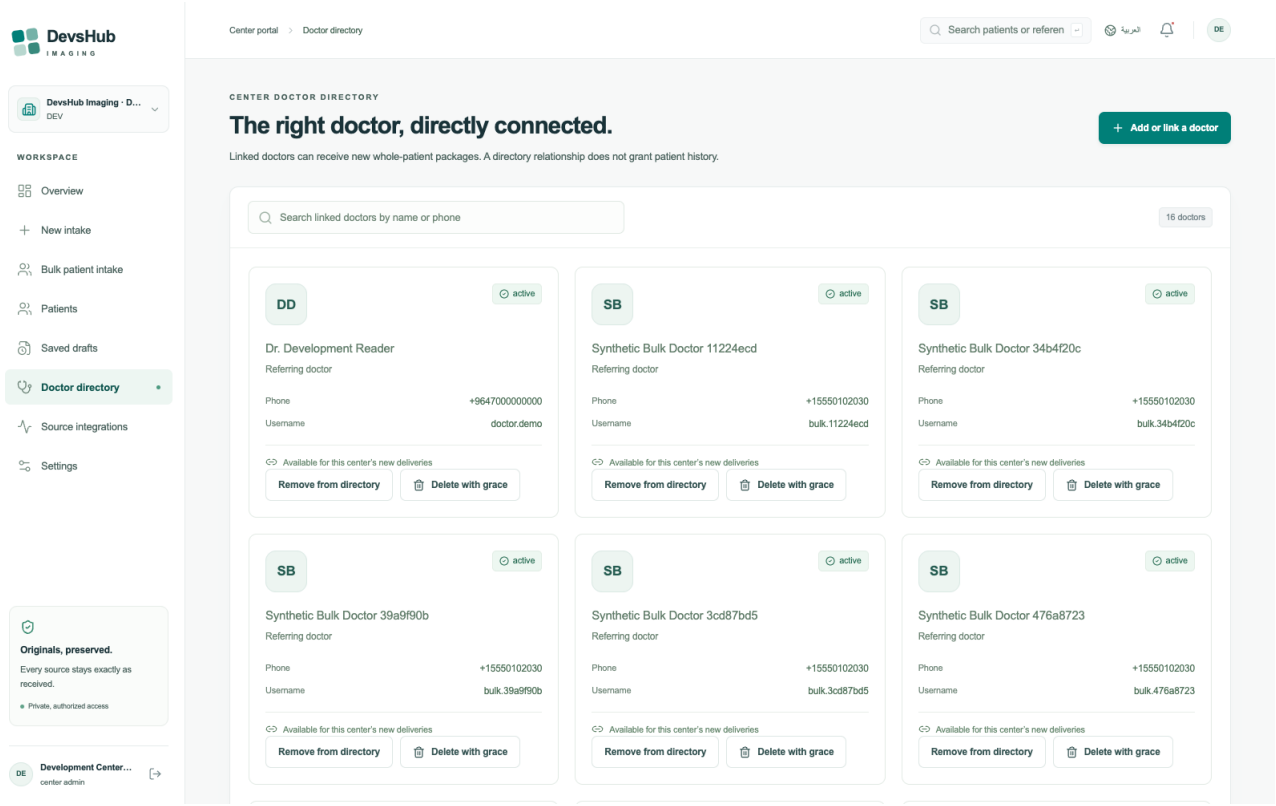
Doctor relationships

A doctor network controlled by the center

The center directory controls which doctors staff can select for new deliveries. Several centers may deliberately work with the same global doctor account, with the sending center clear on every eligible result. A directory relationship makes a doctor selectable; it does not grant patient history.

The platform administrator can enable or disable doctor onboarding for a center owner or examiner. Enabled staff can create a doctor or select an existing active identity without a separate per-doctor approval queue. New accounts require the doctor's name and phone, a unique suggested or chosen username and an initial password the doctor can later change.

A new doctor initially links only to the creating center. Existing linking exposes no password or other-center clinical data. Disabled onboarding still permits sending to already linked, permitted doctors. Removing a directory relationship does not silently rewrite existing examination grants.



The center's selectable doctor directory with demonstration accounts.

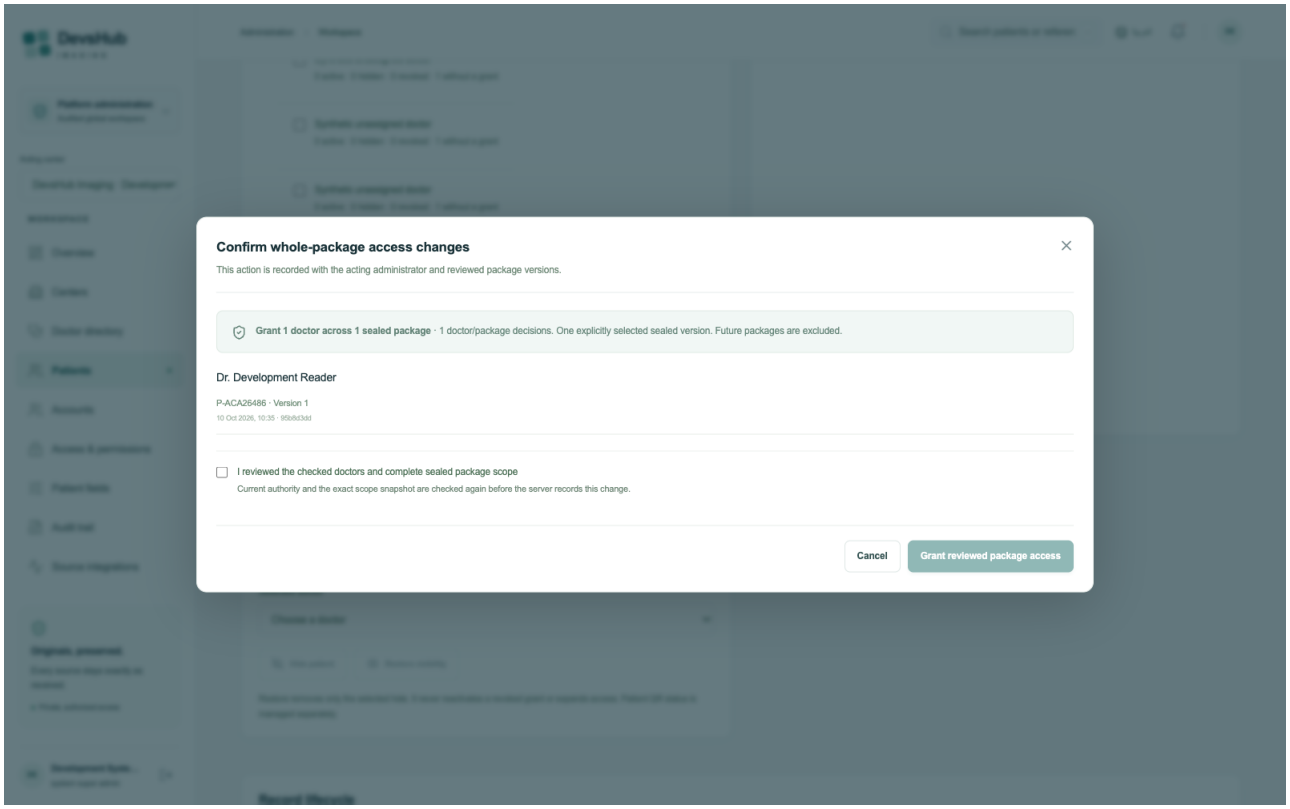
Administration and control

Administration, sharing and clear permissions

The platform's super administrator has audited global control over centers, accounts, patients, files, notes, settings and operations. Additional administrators receive checked actions and selected-center or wider authorized scopes; accounts cannot promote themselves or grant greater rights. Shared-doctor changes must respect every dependent center.

Access managers preview complete packages before granting another doctor all current visits or one exact version. Unmentioned recipients remain unchanged, and future visits are never silently included. Changing the primary doctor explicitly chooses whether to retain or revoke the previous primary doctor's version grant.

Visibility can be hidden for one doctor or all ordinary doctors and remains effective across future visits until restored. Restoration does not revive revoked grants. Doctor hiding is separate from patient QR access; withdrawal, QR revocation and deletion are distinct actions. Permissions remain effective throughout viewing and downloads.



Reviewed package-sharing impact, shown before saving.

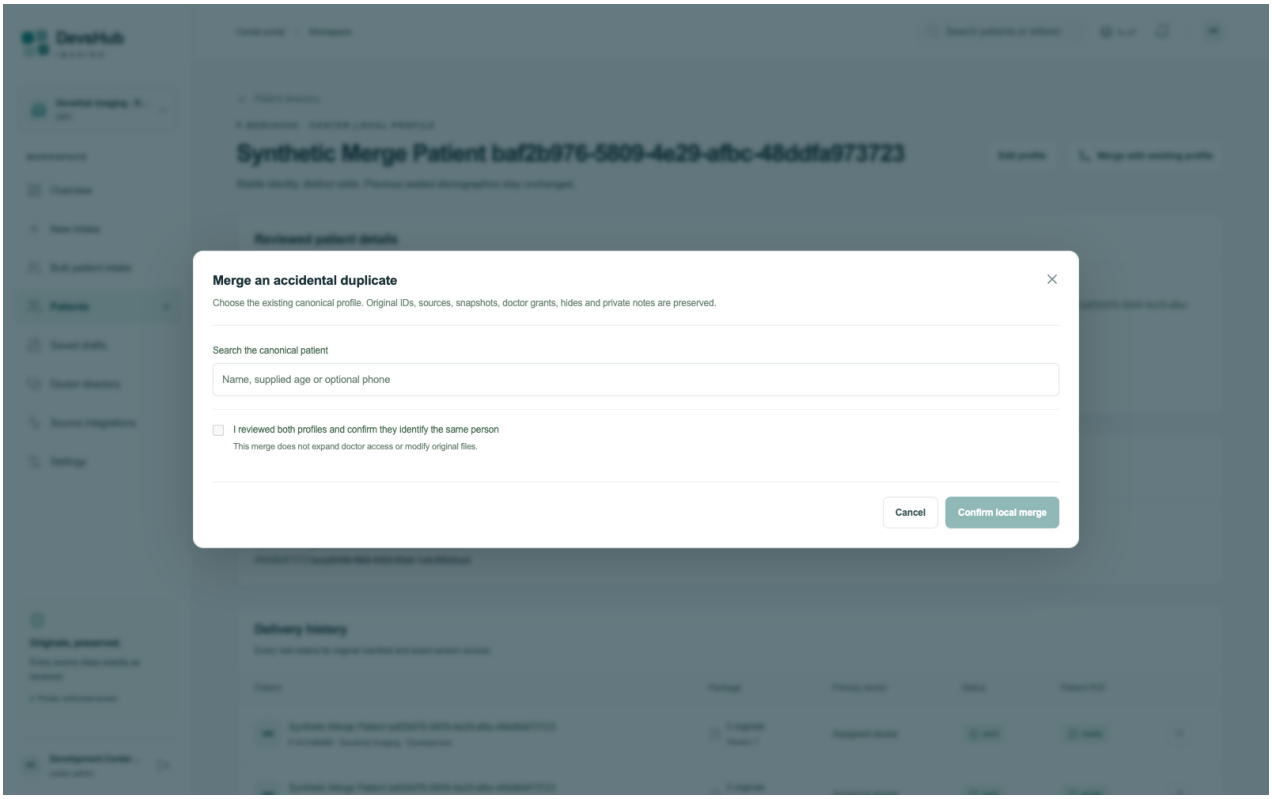
Administration and control

Correct mistakes without losing the history

An explicit same-center merge can reconcile a confirmed duplicate profile while preserving original IDs, sources, visit snapshots, grants and private-note ownership. It never automatically matches names or expands access. Hides remain effective, and an authorized mistaken-merge reversal is available.

Withdrawing a wrong delivery blocks every ordinary recipient and its QR. A new reviewed correction can reuse appropriate retained sources, confirm the correct person and create a new package. Other visits remain untouched; copies already downloaded outside the system cannot be recalled.

Soft deletion blocks ordinary access and allows authorized recovery during the configured grace period. Purge follows the deadline; only the super administrator can request immediate permanent purge. Permanent purge is irreversible, backup retention is separate, and selective erasure protects other patients' retained files. Deleting a doctor does not delete patient examinations. Jobs show processing, failure and governed retry status. Audit history records important actions and their authors.



Explicit duplicate-profile review with demonstration records.

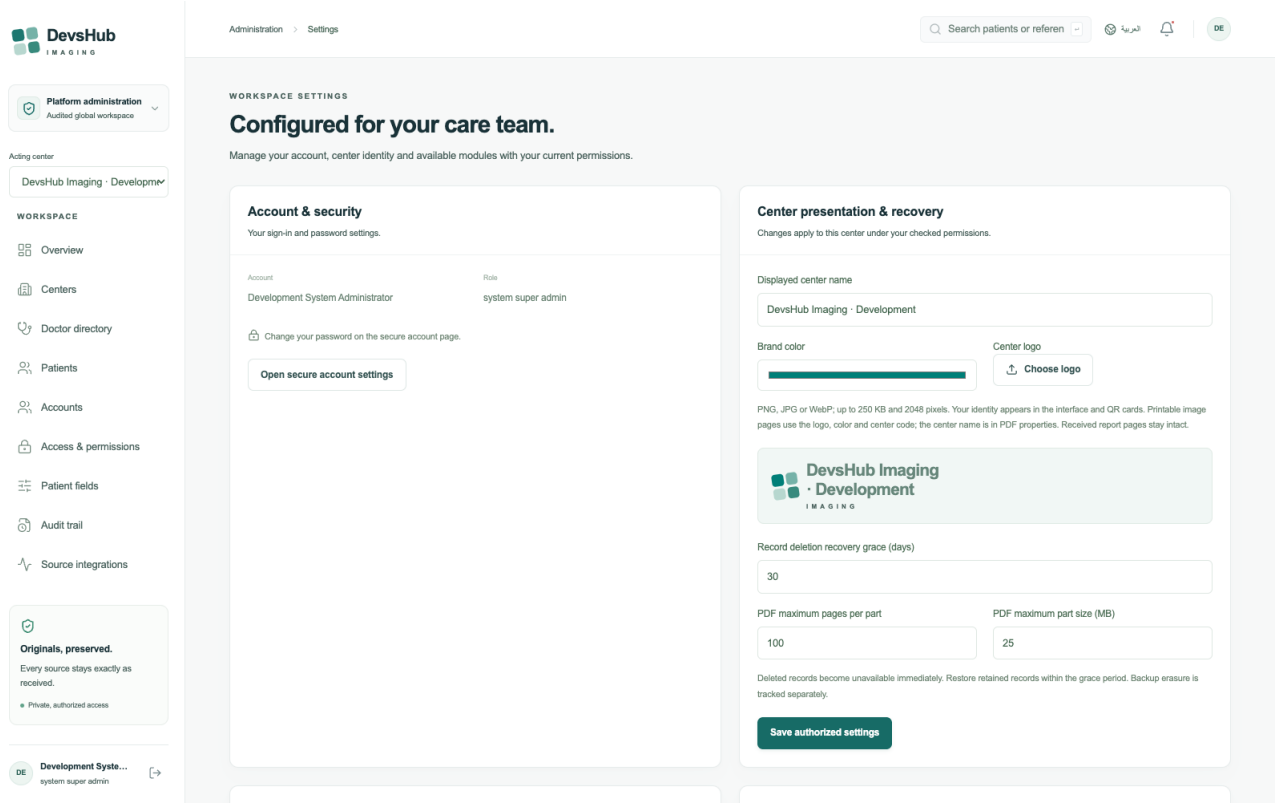
Customization and growth

Fit the product to your center

Private Center provides a dedicated deployment for one center and its linked doctors, with public registration disabled. SaaS supports several isolated centers and self-service registration. Both offer the same product capabilities; an intentionally shared doctor does not combine centers' patient records. Features can be enabled to suit the workflow.

Configure patient field types, order and requiredness, including the default name and age rules. Human identity and whole-source approval always remain mandatory, and historical snapshots stay intact. Arabic/English controls and RTL support do not translate reports, patient values or notes. Center names, validated logos and readable colors personalize the workspace.

Optional Orthanc, DICOMweb, archive-migration and worklist-gateway connections bring sources into ordinary review drafts rather than automatically sending them. Manual upload remains available. Device and gateway formats are reviewed during setup; source files and center boundaries remain protected.



Center presentation and enabled-feature settings.

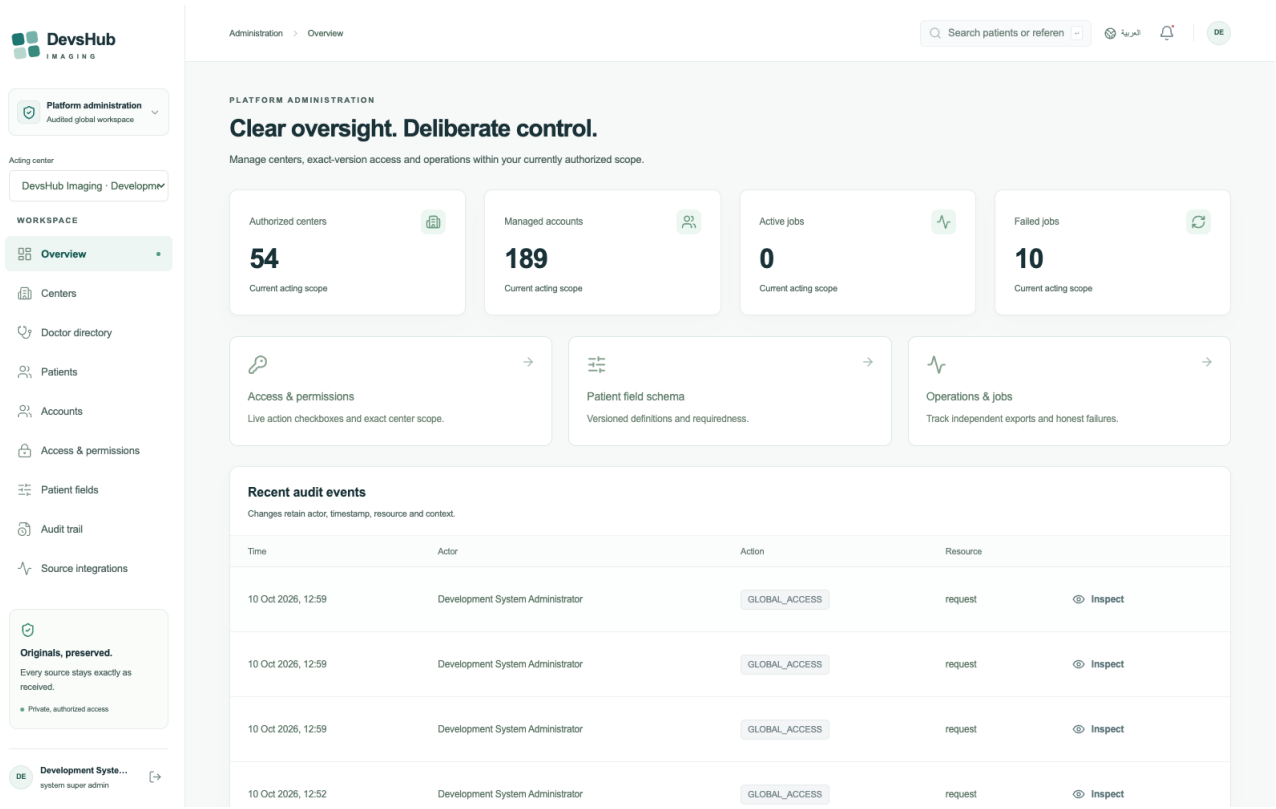
Customization and growth

A platform that grows with your center

Begin with the center and doctors you work with today, then add further centers, users and intentional doctor relationships as the service grows. Dedicated and multi-center options keep the same core workflow: reviewed patients, complete originals, permitted recipients and printable patient results.

Configuration lets administrators adapt fields, appearance, permissions, enabled capabilities and recovery periods without changing the clinical content. Responsive browser access supports everyday work on different screen sizes, while larger screens give doctors more space for detailed imaging. Suitable series and device capabilities determine advanced viewing options.

Saved drafts, resumable transfers, separate printable processing and visible job states help staff manage interruptions without confusing progress with completion. Original preservation and current access remain central throughout. Center setup includes reviewing export formats, doctor relationships, browser needs and operational recovery, so the product fits the actual service being delivered.



Platform oversight with demonstration centers and records.

The complete workflow

Everything in one connected workflow

DevsHub brings the center, doctor and patient journey together while preserving each person's role and the original examination.

- Reviewed DICOM, ZIP and PDF intake; autosaved drafts, resumable transfers and patient-based bulk routing.
- Stable patient profiles, repeat visits, explicit matching, complete source packages and independent printable processing.
- Doctor inbox, eligible history, PDF/DICOM readers, suitable spatial views, original downloads and private dated notes.
- Persistent examination QR, PDF-only patient preview, printing, numbered parts and manual WhatsApp card sharing.
- Doctor onboarding, center directories, scoped administration, reviewed sharing, persistent hiding and audited corrections.
- Merge/reversal, withdrawal, recovery and purge; visible jobs, flexible fields, branding, languages and optional source connections.

Getting started is straightforward: choose a dedicated or multi-center setup, configure your center and permitted doctors, review a patient and their received files, then send. The doctor receives the originals, and the patient receives the printable experience they need.